

US HOSPITAL QUALITY INTELLIGENCE REPORT

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Data Source: CMS Hospital Compare (data.cms.gov)
Hospitals Analyzed: 5,432 across all US states

BUSINESS CONTEXT

Simulated Scenario: Before a critical board review, a regional health network CMO required performance benchmarking. Acting as a Data Analyst, I framed the inquiry around three strategic pillars prior to data extraction, ensuring the transition from standard reactive reporting to high-impact, proactive business intelligence.

KEY FINDINGS

1. The top performing states average 5.8 points above the national average of 8.43, while the bottom performing states average 4.8 points below — representing a 10.6 point performance gap that suggests significant geographical inequality in US hospital quality.
2. 42% of rated US hospitals perform above the national average — but 2,250 hospitals (41% of all facilities) have no CMS rating at all, representing a significant blind spot in our quality assessment.
3. Non-profit hospitals top the average quality score with 9.19 which is 0.76 more than the national average score, followed by For-profit hospitals with 8.14, and hospitals owned by physicians come at the bottom with 4.88 which is 3.55 less than the national average. One noticeable finding is that Tribal owned hospitals have an average score of 7.26, but the total number of tribal owned hospitals is just 16 — insufficient for a reliable conclusion.
4. Even though the number of VA hospitals are very few compared to others, they ranked top with an average quality score of 15.68 — 7.17 points greater than the national average — directly contradicting public perception of VA healthcare quality. Critical Access hospitals score lowest at 3.3, which is 5.3 points below the national average, reflecting structural limitations of rural healthcare infrastructure.
5. No single factor dominates hospital star ratings — the strongest predictor is readmission performance with a correlation of 0.38, while surprisingly safety performance shows almost no relationship with ratings at 0.07, suggesting CMS composite scores balance multiple dimensions of care quality.

RECOMMENDATIONS

1. Network expansion should prioritize MD, MA, NJ partnerships: these states average 5.8 points above national average.
2. Non-profit hospital partnerships offer highest quality ROI: 9.2 average score vs 6.9 for government-owned facilities.
3. Readmission reduction programs yield the highest star rating impact: strongest single predictor at 0.38 correlation.

DATA LIMITATIONS

1. 2,250 hospitals (41%) unrated by CMS.
2. Specialty hospitals excluded from standard quality metrics.
3. US territories removed from geographic analysis.

NEXT STEPS

1. Segment analysis by hospital size and urban/rural classification
2. Time series analysis of quality trends 2020-2024
3. Deep dive into Critical Access hospital performance gap